



POLICY

Custodial Persons and Young Persons in Custody

Scope (Staff):	All Staff
Scope (Area):	CAHS (CACH, CAMHS, PCH, Neonatology and NMHS Security)

Child Safe Organisation Statement of Commitment

CAHS commits to being a child safe organisation by applying the National Principles for Child Safe Organisations. This is a commitment to a strong culture supported by robust policies and procedures to reduce the likelihood of harm to children and young people.

**This policy is currently under review.
The policy content remains current throughout
this review process.**

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Aim

To clearly articulate the management of patients or visitors attending CAHS facilities, who are in the custody of; Western Australia Police, Department of Justice, the Department of Home Affairs (or their contractors).

Risk

Compromised safety of patients, visitors, staff and the general public. Of note, are risks related to aboriginal deaths in custody and child deaths in custody. This policy contains updates in relation to release of information to WA Police in response to coronial recommendations.

Definitions

Custodial Person (visitor): a visitor to a CAHS site or health service who is also not permitted to leave a public or private custodial setting at will by order of any judicial, administrative or other authority.

- This can include a person who is ordered to be detained under the [Prisons Act 1981](#) (WA), [Young Offenders Act 1994](#) (WA), the [Migration Act 1958](#) (Cth), [Crimes Act 1914](#) (Cth), [Customs Act 1901](#) (Cth), Criminal Law (Mentally Impaired Accused) Act 1996 (WA).

Custodial Officer: employee or representative of the agency who escorts the Custodial Person to a health service and takes responsibility for the security, control, safety, care, and welfare and continued custody of the Custodial Person.

Custodial Place: this includes a hospital or other place outside a prison or a detention centre when attended by a custodial person. Perth Children's Hospital (PCH) is deemed a custodial place.

- Refer to the [Court Security and Custodial Services Act 1999 \(WA\)](#) for the list of custodial places.

Escorting Agency: the agency or service that is legally responsible for the security, control, safety, and welfare and continued custody of the custodial person.

Young Person in Custody: (patient or client of CAHS) a person under the age of 18, who is in custody of the Western Australia Police Force, the Department of Justice, Australian Federal Police (AFP) or the Department of Home Affairs who is also receiving treatment or services from CAHS.

Scope for Mental Health Patients

Out of Scope

- Mental health patients admitted under a court ordered Hospital Order are not yet part of the custodial system and fall outside the remit of this policy.

In Scope

- Forensic patients and those on bail can be admitted to the CAMHS Inpatient Unit (IPU). In addition to the processes outlined in this policy, CAMHS staff must also follow:
 - [CAMHS Admission and Care of Mental Health Patients in General Wards](#) procedure
 - [CAMHS Admission to CAMHS Ward 5A](#) procedure
 - CAMHS [Risk Assessment and Management](#) policy.
 - [CAMHS Inpatient Unit Entry Protocol](#)
 - [PCH CAMHS Work Instructions](#)

Scope for Community Health

- Some community health staff visit Bandyup Women's Prison to provide services to the children who are not prisoners but "visitors" who reside at the prison. Therefore, these patients fall outside the scope of the policy.
 - In cases where these children need healthcare a nominated person collects the child from prison and transfers to the appropriate healthcare facility.
 - If the child is transferred to PCH the child is usually accompanied by a nominated relative who is not in custody.
 - If CAHS staff have queries about this process it is best to contact the CAHS Community Health Coordinator of Nursing for advice.

Principles

- A young person in custody has the same right as other people to receive safe, high quality healthcare with respect to their dignity, preferences and special

needs as per the [Charter on The Rights of Children and Young People in Healthcare Services in Australia](#)

- Where this healthcare cannot be provided in the custodial environment the young person in custody may attend CAHS facilities.
- Young people in custody will be;
 - Treated with dignity and respect.
 - Receive information (appropriate to their development) about their health condition.
 - Be involved in their healthcare decision making as much as they wish and are able.
 - Have cultural and personal preferences respected, except where sharing information poses a security risk.
- Young people in custody (and their families), are NOT to receive information about the scheduling or timing of appointments, admission discharge, transfer or movement.
- Normal custodial regulations still apply to young people in custody. This relates to such privileges as; visitors, the young person in custody's contact with the outside world (access to telephones, mail etc.) and freedom of movement.
 - These restrictions will be determined by the relevant Detention Centre or Police Superintendent.
- A Custodial Person or Young Person in Custody attending CAHS sites must always be escorted by the agency / service that is responsible for them under legislative requirements (except where an eligible minimum-security detainee has signed a contract).
 - The proximity of the escort is determined by the security risk of the person in custody and the escorting agencies policies and procedures.
 - The escorting agency (not CAHS staff) will be in control and legally responsible for the continued custody of this person for the duration of the attendance / admission.
- Where a person has been arrested, detained or taken into Police custody, and requires hospitalisation, and / or treatment, the Police service's role is to provide personnel to guard the person at a security level that is appropriate to the individual and the circumstances.
- At no time is a CAHS/ NMHS Security Officer or staff member to be used as a substitute for a Prison Guard or Police Officer.
- All CAHS employees will comply with policy confidentiality requirements regarding Young People in Custody attending CAHS sites in order to assure the safety of staff and visitors.

Process

Notification

- Refer to [Appendix 1](#) for notification process.

Liaison between PCH and Prison Authorities

- QEII MC Security Control Centre is the central contact point between the hospital and the escorting agency for all queries and issues relating to a Custodial Person or Young Person in Custody.
- The escorting agency / service will complete a Security Risk Assessment for all planned admissions / attendances of a Custodial Person / Young Person in Custody at PCH and email it to the QEII MC Security Control Centre.
 - Refer to [Appendix 2: WAPOL Emergency Department Handover](#).
- When an admission is planned, the escorting agency will notify the booking clerk so that the admission related information is updated.
- In emergency presentations the Emergency Department (ED) clerical staff are to be informed of the custodial patient status.
- When an emergency presentation results in an admission, the ED Shift coordinator or Hospital Clinical Manager (HCM) will notify the admitting ward of the custodial patient status.
- The custodial officers will liaise with the onsite security to determine the most suitable path through the hospital when escorting a Custodial Person or Young Person in Custody.
- The escorting agency is responsible for keeping Security Officers and the relevant ward / department informed of any change in circumstances of the Custodial Person or a Young Person in Custody.
- This includes changes to the risk status, surveillance level, the number of Custodial Officers in attendance and rationale for change.
- On the QEII site PCH/ NMHS Security Officers are to be advised by the relevant ward / department Shift Coordinator of the arrival and discharge / departure of a Custodial Person or Young Person in Custody.
- **Minimum security persons** who sign an individual contract with the Department of Justice may be permitted to attend CAHS without custodial officer's present. A copy of this contract must be sent to;
 - the Security Control Centre at NMHS.SecurityControlCentre@health.wa.gov.au.
 - and to the relevant PCH Security Officers at PCH Security Officers PCHSecurityOfficers@health.wa.gov.au.

Management of Custodial Patient Information

HIAS staff

- All Custodial Persons and Young Persons in Custody, both inpatient and outpatient will be given 'SUPPRESSED INFORMATION' status on WebPAS and other CAHS patient information systems (EDIS, CDIS, PSOLIS).
- Young people in the custody of Dept. of Justice (prisoner) or Dept. of Home Affairs (immigration detainee) will have the detention centre superintendent listed as Next of Kin (NOK).
 - Previous NOK history is to be retained but made inactive.

CAHS staff

- Information relating to the presence of Custodial Persons, Young Persons in Custody or Custodial officers on PCH premises is only to be shared with PCH staff on a need-to-know basis. It is not to be provided to any other individuals.

The Patient/Client

- For security reasons, Young Persons in Custody are not to be informed of the time and date of future appointments, admissions, discharge or transfer.
- Young persons in custody can participate as much as they desire and are able in decisions relating to their healthcare and should be given information (as appropriate to their development) to participate in care planning.

Media Statements

- All media statements, verbal or written, relating to a Custodial Person or a Young Person in Custody at PCH will be issued by the escorting agency.
 - This includes patient condition report statements which will be managed by the escorting agency via Child and Adolescent Health Service (CAHS) Communications liaising with PCH clinical staff.
 - Refer to the CAHS [Media Communications](#) policy.

Consent to Treatment of a Young Person in Custody

- See the [CAHS Consent to Treatment Policy](#)
- Where conflict arises about contacting the parent/ legal guardian to consent to treatment and the need to maintain privacy/ security of a young person in custody contact:
 - CAHS Executive Director, Medical Services. For afterhours, consult the Executive on-call.

AND

- the Detention Centre Superintendent OR the Relevant Police Superintendent
- Emergency healthcare can be provided without consent as per the [CAHS consent to Treatment Policy](#)

Restraint

- If a young person in custody requires restraint for treatment/ clinical procedure usual restraint procedures and reporting should be followed. CAHS/ NMHS security may be involved and the restraint is led by clinical staff as per usual policy and procedures.
 - CAHS [Restraint of Patients/ Clients](#) policy.
 - PCH [Clinical Holding](#) Guideline.
 - CAMHS [Physical Restraint](#) policy.
- The custodial officers may assist if their employment enables this action and if the lead clinician agrees.
- In the event the young person is in custody or the custodial is attempting to escape custody it is the role of the custodial officers to manage the continued custody and restraint if necessary, as per their employment guidelines.
- In events where staff, patients and/ or visitors are at risk of violence an aggression [CAHS Code Black](#) procedures are to be followed available for PCH, Community Health and CAMHS.

Clinical Information Sharing at Transitions of Care

- Discharge processes will vary depending on which agency holds custody of the young person in custody.
 - Refer to the following relevant CAHS policies:
 - [Communicating for Safety](#)
 - [Confidentiality, Disclosure and Transmission of Health Information](#)

Dept. of Justice (Prisoners) and Dept. Home Affairs (Immigration Detainees)

- Prisons and Immigration Detention Centres in WA have authorised health service providers and clinical handover can occur according to the WA Health Clinical Handover Policy.
- All medical information relating to the prisoner and immigration detention is to be forwarded to the Medical Bookings Officer at the detention site. This includes:
 - Time and date of future appointments

- Appointment cards
- Letters
- Reports
- Procedure and fasting advice (where relevant).
- Confidential medical information is **not** to be given to the Escorting / Custodial Officer.
- Appendix C for Dept. Justice contacts and Appendix D for Immigration detention contacts

Discharging into the care of WA Police

- Except for the Perth Watch House, Police lockups do not employ medical or nursing clinicians limiting opportunities for clinical handover when discharging into the care of WA Police.
- Where clinical handover cannot occur, and the patient is being discharged into an environment where there is no capacity for medical/ nursing management or supervision the senior discharging clinician should;
 - Request consent from the patient's legal guardian to release some patient personal information to WA Police to support the patients ongoing safety and healthcare.
 - In the absence of a patient's legal guardian and if the patient is deemed a mature minor request consent from the patient to release some relevant personal clinical information to WA Police to support the patient's ongoing safety and healthcare.
 - Where consent cannot be obtained for any reason, the senior discharging clinician is able to release some information pursuant to (S. 220) of the [Health Services \(Information\) Regulations 2017](#) which indicates that the disclosure can occur where it is "reasonably necessary to lessen or prevent a serious risk to the life, health or safety of any individual".
- As a minimum requirement CAHS discharging clinicians provide WA Police with non-personal, written information about signs and symptoms which may indicate a need to re attend hospital. The following non personal documents can be used for this purpose;
 - [The Department of Health Emergency Discharge Information sheets](#),
 - [Procedure specific information sheets](#) (PSIS)
 - [CAHS consumer information sheets](#).
- Where consent cannot be obtained, **and** the senior discharging clinician is unable to disclose patient information under S.220 of the [Health Services \(Information\) Regulations 2017](#);

- Discharge should be reconsidered based on risk to the life, health of the young person in custody.
- Greater than usual consideration should be given to short term admission for young person in custody who may require; serial observations, has complex medical needs or is at risk of clinical deterioration.

Access to Health information on ex-prisoners

- Healthcare providers can request medical information about prisoners through a dedicated telephone line provided by the Department of Justice, Access to Prisoner Information; [Access to health information on ex-prisoners for health providers](#).
 - Ph: 1800 077 735

References and related external legislation, policies, and guidelines
Charter on The Rights of Children and Young People in Healthcare Services in Australia
Court Security and Custodial Services Act 1999 (Western Australian Legislation)
Crimes Act 1914 (Australian Government Federal Register of Legislation)
Criminal Law (Mentally Impaired Accused) Act 1996 (Western Australian Legislation)
Customs Act 1901 (Australian Government Federal Register of Legislation)
DCS Youth Custodial Rule 710 Health Services (Government of Western Australia, Department of Corrective Services)
Health Services Act 2016 (Western Australian Legislation)
Health Services (Information) Regulations 2017 (Western Australian Legislation)
Mental Health Act 2014 (Western Australian Legislation)
Migration Act 1958 (Australian Government Federal Register of Legislation)
Prisons Act 1981 (Western Australian Legislation)
Young Offenders Act 1994 (Western Australian Legislation)
Related CAHS internal policies, procedures and guidelines
Admission and Care of Mental Health Patients in General Wards (PCH CAMHS Policy Manual)
Admission to CAMHS Ward 5A (PCH CAMHS Policy Manual)

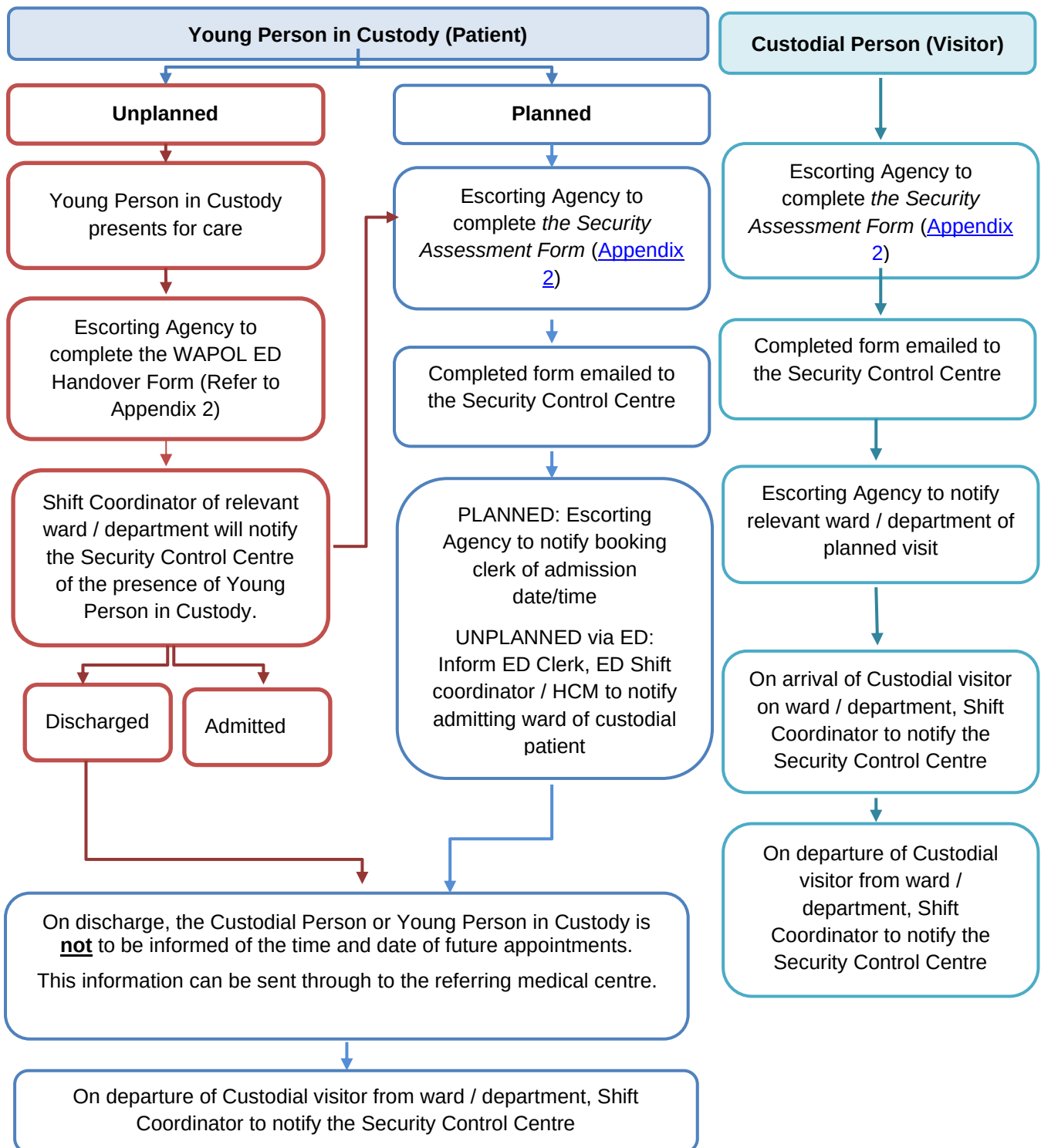
CAHS Emergency Code Procedures
CAMHS Inpatient Unit (IPU) Entry Protocol (PCH CAMHS Policy Manual)
Clinical Holding (PCH Clinical Practice Manual)
Communicating for Safety (CAHS Policy Manual)
Confidentiality, Disclosure and Transmission of Health Information (CAHS Policy Manual)
Consent to Treatment (CAHS Policy Manual)
Media Communications (CAHS Policy Manual)
Physical Restraint (PCH CAMHS Policy Manual)
Restraint of Non-Patients / Clients (CAHS Policy Manual)
Risk Assessment and Management (CAMHS Policy Manual)

Useful resources (including related forms)
Department of Justice; Access to health information on ex-prisoners for health providers
Procedure specific information sheets (PSIS)
Security Risk Assessment (Available from Security)
The Department of Health Emergency Discharge Information sheets (CAHS ED)

This document can be made available in alternative formats on request.

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Appendix 1: Process for Notification of Custodial Persons



Appendix 2: WAPOL Emergency Department Handover

Having fully considered the information set out in the *Details of Presentation*, *Police Handover to Hospital Staff* and *History* sections of this form, **if the information gives rise to a reasonable belief that the person is likely to pose a risk to the safety of other persons (including hospital personnel and/or other members of the public), the following is to apply:**

Category A - Persons in lawful custody charged with an offence where bail is to be refused, or has been refused:

- Police are to remain until such time as a bedside hearing occurs (if applicable) or such time as the person is discharged from hospital in custody of police (whichever occurs first).
- If, having appeared at a bedside court hearing, the person is released on bail, the procedures set out in **Category E** applies.
- If, having appeared at a bedside hearing, the person is Remanded in Custody – police are to remain until such time as custody of the person is transferred to the Department of Corrective Services officers (or applicable custodial service provider officers).

Category B – Persons in lawful custody charged with an offence where bail is to be granted:

- Police are to remain until such time as the person is released on bail or such time as the person is discharged from hospital in custody of police (whichever occurs first).
- If, having being taken to hospital and subsequently released on bail by an Authorised Police officer, the procedures set out in **Category E** applies.

Category C – Persons in lawful custody pursuant to an Arrest Warrant / Warrant of Commitment

- Unless approval has been granted not to execute the Warrant in line with WA Police Force procedures, Police are to remain until such time as custody of the person is handed over to Department of Corrective Services officers (or applicable custodial service provider officers).

Category D – Persons detailed under the *Mental Health Act 2014*

- Note: Under Section 156 of the *Mental Health Act 2014*, Police are only authorised to detain a person (apprehended under this section) until such time as the person is –
 - Received into the place where the assessment will be conducted, or;
 - The person is delivered into the care of the authorised medical practitioner who will assess the person.

- In these circumstances, Police will remain for a reasonable time to enable the hospital to organise the attendance of security personnel and/or medical intervention which mitigates the risk that the person poses to the safety of other persons.

Category E – Persons not in Police custody (e.g. witnesses / victims / other persons not in police custody)

- Note: Police have no authority to detain a person unless that person is in lawful custody pursuant to legislation.
- Where, having conveyed a person to hospital who is not in lawful custody, Police form the view there is a reasonable belief that the person is likely to pose a risk to the safety of other persons, (including hospital personnel and/or other members of the public), Police will remain for a reasonable time to enable the Hospital to put in place suitable arrangements.
 - This may include attendance of Hospital Security personnel and/or some other arrangement (such as medical intervention) which mitigates the risk that the person poses to the safety of other persons.

WAPOL ED Handover

Date:		Time:	
Perth Children's Hospital EMERGENCY DEPARTMENT POLICE INFORMATION FORM		PATIENT LABEL	
Police Regimental numbers:		Police Station / Business area:	
		Phone Number:	
DETAILS OF PRESENTATION			
Describe the circumstances and time of incident:			
Why has the person been brought to hospital? ☐ Detainee – In Custody – Medical Examination / Treatment ☐ Detainee – In Custody – Executed Warrant – Medical Examination / Treatment ☐ Detainee – <i>Mental Health Act 2014</i> ☐ Other Police related (In care but not Custody) – Medical Examination / Treatment <i>Note: Definitions on page 4</i>			
Details of incident: (Please provide relevant to assist in assessing the risk posed to hospital staff) Person aggressive: ☐ Yes ☐ No Describe behaviours exhibited during incident: ☐ Mental Health episode suspected ☐ Section 156 MHA used? ☐ Attempted self-harm ☐ Appears drug and/or alcohol affected. ☐ Threatened another with weapons ☐ Person assaulted another ☐ Threats to kill / seriously injure made ☐ Restraints used during arrest to control subject ☐ Taser / OC / Baton drawn or deployed during arrest ☐ Police Firearm drawn during arrest ☐ Patient or police received injuries during incident ☐ Found in possession of weapons Other & relevant comments:			

WAPOL ED Handover

POLICE HANDOVER TO HOSPITAL STAFF	
Has the person been searched? ☐ Yes ☐ No Level of search (eg basic / strip-search): Were any high-risk items (eg weapons / drugs) located in the person's possession? ☐ Yes ☐ No If yes, provide details: Have any high-risk items been left in the person's possession: ☐ Yes ☐ No If yes, provide details:	
Has property (other than high risk items) been removed from the person? ☐ Yes ☐ No If yes, please list what has been removed and where the items have been stored:	

HISTORY		
Has the person displayed any recent violent or threatening behaviour that may indicate there is a risk to hospital staff?	☐ Yes ☐ No	Detail:
Does the person have any Warnings, Alerts or history for any of the following? May Assault Police May attempt self-harm Claims to have an infectious disease Any other matter that reasonably gives rise to believe the person may endanger another person's safety	☐ Yes ☐ No ☐ Yes ☐ No ☐ Yes ☐ No ☐ Yes ☐ No	Detail:
Are there any other matters known by attending officers which give rise to a reasonable belief that the person is likely to pose a risk to the safety of other persons?	☐ Yes ☐ No	Detail:

CLINICIAN USE		
Does the patient have an ED Management Plan (EDMP)? ☐ Yes ☐ No If not, do they require an EDMP? ☐ Yes ☐ No	Identified in PSOLIS as an aggression risk? ☐ Yes ☐ No	WebPAS aggression flag? ☐ Yes ☐ No ☐ N/A

WAPOL ED Handover

HOSPITAL STAFF TO COMPLETE WITH POLICE

RISK ASSESSMENT MATRIX - Please indicate patient behavioural risk level

	Low	Medium	High	Extreme
	Abuse, swearing directed at police, hospital staff or general public Non-specific threat to injure self or another. Heated disagreement raised voices.	Assault of another resulting in minor injury. Includes pushing grabbing, scratching. Specific threat to injure self or another. Overtly physically aggressive but assault has not occurred.	Threatened use of weapon or significant force (AOBH) Biting, spitting and resisting arrest or assaulting police Includes force / threats to commit sexual offences. Possession of weapons	Actual use of weapon or significant force (serious AOBH, GBH or death) Extreme aggression towards police / authority
Behaviour Prior to and During Police Custody	☐	☐	☐	☐
History (Criminal and Suspected)	☐	☐	☐	☐
Overall Risk	☐	☐	☐	☐

NOTE: Highest risk is overall risk.

RISK ASSESSMENT ACTION PLAN – to be completed in conjunction with WAPOL and ED Clinician

Risk Rating	Action Plan	Signed
EXTREME RISK	- Police to remain with patient if in police custody. - If not in police custody, police to remain a reasonable time to enable the Hospital to put in place suitable security arrangements. - Consider alternative options for medical care	
HIGH RISK	- Police to remain with patient if in police custody. - If not in police custody, police to remain a reasonable time to enable the Hospital to put in place suitable security arrangements.	
MEDIUM RISK	- Police to remain with patient if in police custody. - Security advised and on standby	
LOW RISK	Patient management as per clinical/behavioural requirements	

Document changes to Risk Assessment Action Plan in the patient integrated notes

RISK ASSESSMENT AND ACTION PLAN AGREEMENT

Clinician's name:	Police Officer's name:
Signature: Date:	Signature: Date: